

**Laboratory Investigation Report**

Patient Name	: Tuba	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 24 Y 7 M 1 D /F	OP/IP No	: OP/99812
Max ID/Mobile	: 333432/8752005200	Collection Date/Time	: 31/Aug/2021 12:51PM
Lab ID	: 1266082100725	Receiving Date	: 31/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 01/Sep/2021

Serology Elisa**IgE (Immunoglobulin-E)*, Serum**

FEIA

Date	31/Aug/2021 12:51PM	Unit	Bio Ref Interval
Immunoglobulin-IgE	141	kUA/L	0-64

Comment Total IgE is an in vitro test system for the quantitative measurement of circulating total IgE in human serum or plasma. It is intended for in vitro diagnostic use as an aid in the clinical diagnosis of IgE mediated allergic disorders in conjunction with other clinical findings, and is to be used in clinical laboratories. A definite clinical diagnosis should not be made as a result of single test only, but should be made by taking into account clinical history and other laboratory findings.

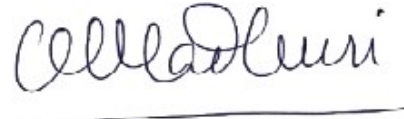
Kindly correlate with clinical findings

***** End Of Report *****

Dr. Bansidhar Tarai, M.D.
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Dr. Madhuri Somani, M.D., DNB
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SIN No: B2B907048, Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017
Booking Centre : 1534 - Arctern Healthcare Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752005200

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Max Lab, Max Hospital, Gurgaon: Opposite HUDA City Centre Metro Station, B-Block, Sushant Lok-1, Gurgaon-122001,
Phone: +91-124-6623 000 | (CIN No.: U85100DL2021PLC381826)

Helpline No. 7982 100 200 | www.maxlab.co.in | feedback@maxlab.co.in

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Clinical Biochemistry

Test Name	Result	Unit	Bio Ref Interval
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IgA (Immunoglobulin-A)*, Serum

Immunoglobulin IgA Immunoturbidimetric	131.84	mg/dL	70 - 400
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Interpretation

Polyclonal-increased seen in chronic inflammatory disorders, infectious processes and in association with tumors, rheumatoid arthritis, inflammatory bowel disease, ankylosing spondylitis, mixed connective tissue disease.

Monoclonal-increased seen in multiple myeloma, solitary plasmacytoma, α -heavy chain disease, monoclonal gammopathy of undetermined significance (MGUS), Lymphoma and Chronic Lymphocytic leukemia.

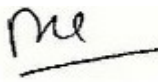
Decreased in primary immunodeficiencies associated with infectious, autoimmune and allergic complications, gastric carcinoma and lymphoma.

Kindly correlate with clinical findings

*** End Of Report ***



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Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
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Test Name	Outsourced Result	Unit	Bio Ref Interval
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MTHFR Gene Mutation PCR (L)*, EDTA

Real Time PCR

MTHFR (Methyl Tetrahydrofolate Reductase) GENE MUTATION

C677T	Heterozygous mutation detected
A1298C	Heterozygous mutation detected

Ref Range

Result	Comments
Homozygous mutation detected	Both copies of the gene carry mutation.
Heterozygous mutation detected	One copy of the gene carries mutation.
Not Detected	Mutation not detected.

Note

1. This assay detects the mutation C677T and A1298C in MTHFR.
2. This is an in-house developed assay
3. Test conducted on Whole blood
4. Genetic counseling available.

Comments

Homozygous mutation for MTHFR (Methyltetrahydrofolate reductase) is associated with hyperhomocysteinemia which is an independent risk factor for Stroke, Myocardial infarction, Peripheral arterial disease and venous thrombosis. Indian studies suggest that heterozygosity for MTHFR C 677T is also associated with elevated homocysteine levels. MTHFR C677T or A1298C carriers are not at increased risk for thrombosis in the absence of hyperhomocysteinemia. Homozygous MTHFR C 677T or A1298C carriers are at increased risk for hyperhomocysteinemia if they become deficient in vitamins B 6, B12 or folic acid. Hyperhomocysteinemia is a relatively weak risk factor for both venous thromboembolism and arterial thrombosis. According to ACMG guidelines there is growing evidence that MTHFR polymorphism testing has minimal clinical utility and, therefore should not be ordered as a part of a routine evaluation for thrombophilia.



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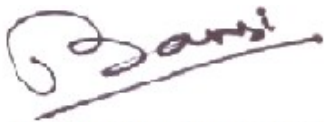
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Test Name	Outsourced Result	Unit	Bio Ref Interval
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Note : Outsourced Test

Kindly correlate with clinical findings

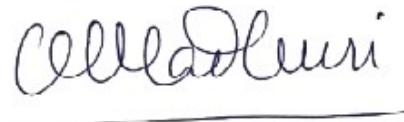
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Hematology Special**Natural Killer Cells, Flow Cytometry, EDTA****Flow Cytometry Number**

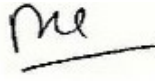
Total Leucocytes	5600	cells/ μ l
Lymphocyte percentage	27.5	%
NK cells percentage:	9.59	% of gated lymphocytes
NK cells absolute count	147.62	cell/ μ l
(Expected reference range 67 - 1134 cells/ μ l)		

IM 141/2021

Kindly correlate with clinical findings

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